



MC-7005

PATIENT NAME : MRS. AMBILY ARUN

REF. DOCTOR : SELF

ACCESSION NO : **4126YL001618**

AGE/SEX : 48 Years Female

PATIENT ID : AMBIF1006774126

DRAWN : 05/12/2025 08:45:30

CLIENT PATIENT ID:

RECEIVED : 05/12/2025 08:47:25

ABHA NO :

REPORTED : 05/12/2025 11:38:00

Test Report Status **Final**

Results

Units

HAEMATOLOGY

WELL WOMAN BASIC PACKAGE 2 (FBS)**GLYCOSYLATED HEMOGLOBIN(HBA1C), EDTA WHOLE BLOOD**

HBA1C	6.3	Normal	: 4.0-5.6%.
		Non-diabetic level	: <5.7%.
		Diabetic	: >6.5%

Glycemic control goal
 More stringent goal : <6.5%.
 General goal : <7%.
 Less stringent goal : <8%.

Glycemic targets in CKD :-
 If eGFR > 60 : <7%.
 If eGFR < 60 : 7-8.5%.

METHOD : HPLC

ESTIMATED AVERAGE GLUCOSE(EAG)	134.1 High	< 116.0	mg/dL
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BLOOD COUNTS, EDTA WHOLE BLOOD

HEMOGLOBIN	12.4	12.0 - 15.0	g/dL
METHOD : PHOTOMETRIC METHOD BY SLS HAEMOGLOBIN			
RED BLOOD CELL COUNT	4.69	3.80 - 4.80	mil/ μ L
METHOD : HYDRODYNAMIC FOCUSING BY DC DETECTION			
WHITE BLOOD CELL COUNT	7.83	4.0 - 10.0	thou/ μ L
METHOD : FLOWCYTOMETRY ,IMPEDENCE AND ABSORBENCY			
PLATELET COUNT	334	150 - 410	thou/ μ L
METHOD : HYDRODYNAMIC FOCUSING BY DC DETECTION			

RBC AND PLATELET INDICES

HEMATOCRIT	38.3	36.0 - 46.0	%
METHOD : RBC PULSE HEIGHT DETECTION			
MEAN CORPUSCULAR VOLUME (MCV)	81.7 Low	83.0 - 101	fL
METHOD : CALCULATED			
MEAN CORPUSCULAR HEMOGLOBIN (MCH)	26.4 Low	27.0 - 32.0	pg

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CONSULTANT BIOCHEMIST
DR.NISHA G, MBBS MD(PATH),
(Reg No - TCMC:45399)
CONSULTANT PATHOLOGIST

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 ERNAKULAM, 682036
 KERALA, INDIA
 Tel : 93334 93334
 Email : customercare.ddrc@agilus.in



Patient Ref. No. 666000016385440



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METHOD : CALCULATED			
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION (MCHC)	32.4	31.50 - 34.50	g/dL
METHOD : CALCULATED			
RED CELL DISTRIBUTION WIDTH (RDW)	13.8	11.60 - 14.0	%
METHOD : CALCULATED			
MEAN PLATELET VOLUME (MPV)	10.7	6.80 - 10.90	fL
METHOD : CALCULATED			
WBC DIFFERENTIAL COUNT			
SEGMENTED NEUTROPHILS	56	40.0 - 80.0	%
METHOD : FLOW CYTOMETRY			
LYMPHOCYTES	26	20.0 - 40.0	%
METHOD : FLOW CYTOMETRY			
MONOCYTES	5	2.0 - 10.0	%
METHOD : FLOW CYTOMETRY			
EOSINOPHILS	13 High	1.0 - 6.0	%
METHOD : FLOW CYTOMETRY			
BASOPHILS	0	0.0 - 1.0	%
METHOD : FLOW CYTOMETRY			
ABSOLUTE NEUTROPHIL COUNT	4.38	2.0 - 7.0	thou/ μ L
METHOD : FLOW CYTOMETRY			
ABSOLUTE LYMPHOCYTE COUNT	2.03	1.0 - 3.0	thou/ μ L
METHOD : FLOW CYTOMETRY			
ABSOLUTE MONOCYTE COUNT	0.41	0.20 - 1.0	thou/ μ L
METHOD : FLOW CYTOMETRY			
ABSOLUTE EOSINOPHIL COUNT	0.99 High	0.02 - 0.50	thou/ μ L
METHOD : FLOW CYTOMETRY			
ABSOLUTE BASOPHIL COUNT	0.00	0.00 - 0.10	thou/ μ L

ERYTHROCYTE SEDIMENTATION RATE (ESR), WHOLE BLOOD

SEDIMENTATION RATE (ESR)	12	0 - 20	mm at 1 hr
METHOD : WESTERGREN METHOD			

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CONSULTANT BIOCHEMIST

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BIO CHEMISTRY**WELL WOMAN BASIC PACKAGE 2 (FBS)****CALCIUM, SERUM**

CALCIUM

9.5

18-60 yrs : 8.6-10

mg/dL

METHOD : NM-BAPTA

CREATININE, SERUM

CREATININE

0.87

18 - 60 yrs : 0.6 - 1.1

mg/dL

METHOD : KINETIC COLORIMETRIC JAFFE

GLUCOSE FASTING, FLUORIDE PLASMA

FBS-FASTING BLOOD SUGAR (GLUCOSE)

125

 Diabetes Mellitus : > or = 126 mg/dL
 Impaired fasting Glucose/
 Prediabetes : 101 - 125.
 Hypoglycemia : < 55.

METHOD : HEXOKINASE ENZYMATIC

IRON, SERUM

IRON

83

Adult (Female) : 37-145

µg/dL

METHOD : FEROZINE

ASPARTATE AMINOTRANSFERASE (AST/SGOT), SERUMASPARTATE AMINOTRANSFERASE
(AST/SGOT)

23

Adults : < 33

U/L

METHOD : IFCC KINETIC WITH P5P

ALANINE AMINOTRANSFERASE (ALT/SGPT), SERUM
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ALANINE AMINOTRANSFERASE (ALT/SGPT)

18

Adults : < 34

U/L

METHOD : IFCC KINETIC WITH P5P

ALKALINE PHOSPHATASE, SERUM

ALKALINE PHOSPHATASE

61

Adult (<60yrs) : 35 - 105

U/L

METHOD : IFCC PNP AMP BUFFER

TOTAL PROTEIN, ALBUMIN, GLOBULIN, SERUM

TOTAL PROTEIN

7.3

Ambulatory : 6.4 - 8.3

g/dL

Recumbant : 6 - 7.8

METHOD : BIURET

ALBUMIN

4.5

20-60yrs : 3.5 - 5.2

g/dL

METHOD : BCG

GLOBULIN

2.8

General Range : 2 - 3.5

g/dL

Premature Neonates : 0.29 - 1.04

METHOD : CALCULATED PARAMETER

ALBUMIN/GLOBULIN RATIO

1.6

General Range : 1.1 - 2.5

Ratio

METHOD : CALCULATED PARAMETER

BILIRUBIN (TOTAL, DIRECT, INDIRECT), SERUM

BILIRUBIN, TOTAL

0.29

General Range : < 1.1

mg/dL

METHOD : COLORIMETRIC DIAZO METHOD

BILIRUBIN, DIRECT

0.17

General Range : < 0.3

mg/dL

METHOD : COLORIMETRIC DIAZO METHOD

BILIRUBIN, INDIRECT

0.12

General Range : <0.85

mg/dL

METHOD : DIAZO METHOD

PHOSPHORUS, SERUM

PHOSPHORUS

4.1

> 18 yrs : 2.5-4.5

mg/dL

METHOD : ENZYMATIC PHOSPHOMOLYBDATE

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POTASSIUM, SERUM

POTASSIUM, SERUM

4.76

Adults : 3.5-5.1

mEq/L

METHOD : ISE INDIRECT

SODIUM, SERUM

SODIUM, SERUM

135

18-90 yrs : 136-145

mEq/L

METHOD : ISE INDIRECT

UREA

BLOOD UREA NITROGEN

10

Adult(<60 yrs) : 6 to 20

mg/dL

METHOD : CALCULATED

UREA

21.4

18-60 yrs : 12.85-42.83

mg/dl

METHOD : UV KINETIC (USING UREASE/GLDH)

URIC ACID, SERUM

URIC ACID

5.1

Adults : 2.4-5.7

mg/dL

METHOD : URICASE ENZYMATIC WITH ASCORBATE OXIDASE

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NEPHELOMETRY

RHEUMATOID FACTOR QUANTITATIVE, SERUM

RHEUMATOID FACTOR

<8.3

Cut-off : < 14

IU/mL

METHOD : IMMUNOTUBIDIMETRY

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BIOCHEMISTRY - LIPID

WELL WOMAN BASIC PACKAGE 2 (FBS)**LIPID PROFILE, SERUM**

CHOLESTEROL, TOTAL

313 High

Desirable : < 200 mg/dL
 Borderline : 200-239
 High : >or= 240

METHOD : CHOD - PAP

TRIGLYCERIDES

155 High

Normal : < 150 mg/dL
 High : 150-199
 Hypertriglyceridemia : 200-499
 Very High : > 499

METHOD : GPO - PAP

HDL CHOLESTEROL

62

General range : 40-60 mg/dL

METHOD : HOMOGENOUS DIRECT ENZYMATIC COLORIMETRIC

LDL CHOLESTEROL, DIRECT

229 High

Optimum : < 100 mg/dL
 Above Optimum : 100-129
 Borderline High : 130-159
 High : 160-189
 Very High : >or= 190

METHOD : HOMOGENEOUS ENZYMATIC COLORIMETRIC ASSAY

NON HDL CHOLESTEROL

251 High

Desirable : < 130 mg/dL
 Above Desirable : 130 -159
 Borderline High : 160 - 189
 High : 190 - 219
 Very high : > / = 220

METHOD : CALCULATED PARAMETER

VERY LOW DENSITY LIPOPROTEIN

31.0

</= 30.0 mg/dL

METHOD : CALCULATED PARAMETER

CHOL/HDL RATIO

5.1 High

Low Risk : 3.3 - 4.4
 Average Risk : 4.5 - 7.0
 Moderate Risk : 7.1 - 11.0
 High Risk : > 11.0

LDL/HDL RATIO

3.7 High

Desirable/Low Risk - 0.5-3
 Borderline/Moderate Risk- 3.1-6
 High Risk- >6.0

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Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A. CAD with > 1 feature of high risk group
	B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease
Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

THYROID PANEL II (FT3,FT4,TSH) SERUM

FREE TRIIODOTHYRONINE (FT3)

2.64

Adult : 2.0-4.4

pg/mL

Pregnant : 2.0-3.8

METHOD : ECLIA

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FREE THYROXINE (FT4)

1.11

Adult : 0.93-1.7

ng/dL

Pregnant (Trimester-wise) :

1st : 0.7-2.0

2nd : 0.5-1.6

3rd : 0.5-1.6

METHOD : ECLIA

TSH (ULTRASENSITIVE)

3.710

Non-Pregnant : 0.4-4.2

μIU/mL

Pregnant Trimester-wise :

1st : 0.1 - 2.5

2nd : 0.2 - 3

3rd : 0.3 - 3

METHOD : ECLIA

Interpretation(s)

Sr. No.	TSH	FT4	FT3	Possible Conditions
1	High	Low	Low	(1) Primary Hypothyroidism (2) Chronic autoimmune Thyroiditis (3) Post Thyroidectomy (4) Post Radio-Iodine treatment
2	High	Normal	Normal	(1) Subclinical Hypothyroidism (2) Patient with insufficient thyroid hormone replacement therapy (3) In cases of Autoimmune/Hashimoto thyroiditis (4). Isolated increase in TSH levels can be due to Subclinical inflammation, drugs like amphetamines, Iodine containing drug and dopamine antagonist e.g. domperidone and other physiological reasons.
3	Normal/Low	Low	Low	(1) Secondary and Tertiary Hypothyroidism
4	Low	High	High	(1) Primary Hyperthyroidism (Graves Disease) (2) Multinodular Goitre (3) Toxic Nodular Goitre (4) Thyroiditis (5) Over treatment of thyroid hormone (6) Drug effect e.g. Glucocorticoids, dopamine, T4 replacement therapy (7) First trimester of Pregnancy
5	Low	Normal	Normal	(1) Subclinical Hyperthyroidism
6	High	High	High	(1) TSH secreting pituitary adenoma (2) TRH secreting tumor
7	Low	Low	Low	(1) Central Hypothyroidism (2) Euthyroid sick syndrome (3) Recent treatment for Hyperthyroidism

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8	Normal/Low	Normal	High	(1) T3 thyrotoxicosis (2) Non-Thyroidal illness
9	Low	High	Normal	(1) T4 Ingestion (2) Thyroiditis (3) Interfering Anti TPO antibodies

REF: 1. TIETZ Fundamentals of Clinical chemistry 2.Guidlines of the American Thyroid association during pregnancy and Postpartum, 2011.

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SPECIALISED CHEMISTRY - HORMONE

TESTOSTERONE, TOTAL, SERUM

TESTOSTERONE, TOTAL

20.60

<50yrs : 8.4-48.1

ng/dl

METHOD : ECLIA

ESTRADIOL (E2), SERUM

ESTRADIOL

19.2

Menstrual phase based:

pg/mL

Early Follicular : 20-150

Late Follicular : 40-350

Midcycle : 150-750

Luteal : 30-450

Pregnant (Trimester-wise):

1st : 154-3243

2nd : 1561-21280

3rd : 8525- >30000

Post-menopausal :-

<21

METHOD : ECLIA

PROGESTERONE, SERUM

PROGESTERONE

0.33

Menstrual phase based :- ng/mL

Follicular : 0.057-0.893

Ovulation : 0.121-12

Luteal : 1.83-23.9

Pregnancy (Trimester-wise) :-

1st : 11-44.3

2nd : 25.4-83.3

3rd : 58.7-214

Postmenopausal : <0.126

METHOD : ECLIA

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PATIENT NAME : MRS. AMBILY ARUN

REF. DOCTOR : SELF

ACCESSION NO : **4126YL001618**

PATIENT ID : AMBIF1006774126

CLIENT PATIENT ID:

ABHA NO :

AGE/SEX : 48 Years Female

DRAWN : 05/12/2025 08:45:30

RECEIVED : 05/12/2025 08:47:25

REPORTED : 05/12/2025 11:38:00

Test Report Status **Final**

Results

Units

CLINICAL PATH - URINALYSIS

WELL WOMAN BASIC PACKAGE 2 (FBS)**PHYSICAL EXAMINATION, URINE**

COLOR PALE YELLOW

METHOD : MACROSCOPIC EXAMINATION

APPEARANCE CLEAR

METHOD : MACROSCOPIC EXAMINATION

CHEMICAL EXAMINATION, URINE

PH 6.5 4.8 - 7.4

METHOD : REACTION OF INDICATORS WITH H⁺ - IONS

SPECIFIC GRAVITY 1.015 1.015 - 1.030

METHOD : CHANGE IN ION CONCENTRATION, CHANGES THE COLOUR INDICATOR THROUGH A COMPLEXING AGENT

PROTEIN NOT DETECTED NOT DETECTED

METHOD : PROTEIN ERROR OF PH INDICATOR

GLUCOSE NOT DETECTED NOT DETECTED

METHOD : GOD-POD METHOD

KETONES NOT DETECTED NOT DETECTED

METHOD : PRINCIPLE OF LEGAL'S TEST

BLOOD NOT DETECTED NEGATIVE

METHOD : PEROXIDASE- LIKE ACTION OF HB CATALYZES THE OXIDATION OF INDICATOR

BILIRUBIN NOT DETECTED NOT DETECTED

METHOD : COUPLING OF BILIRUBIN WITH DIAZOTIZED

UROBILINOGEN NORMAL NORMAL

METHOD : REACTION OF STABLE DIAZONIUM SALT

NITRITE NOT DETECTED NOT DETECTED

METHOD : GRIESS TEST

LEUKOCYTE ESTERASE NOT DETECTED NOT DETECTED

METHOD : REACTION BY ACTION OF ESTERASE

MICROSCOPIC EXAMINATION, URINE

RED BLOOD CELLS NOT DETECTED NOT DETECTED /HPF

METHOD : MICROSCOPIC EXAMINATION

PUS CELL (WBCS) 2-3 0-5 /HPF

DR. NISHA G, MBBS MD(PATH),
(Reg No - TCMC:45399)
CONSULTANT PATHOLOGIST

DR. DEVI C B MBBS, MD
CONSULTANT BIOCHEMIST

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ERNAKULAM, 682036
KERALA, INDIA
Tel : 93334 93334
Email : customercare.ddrc@agilus.in


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METHOD : MICROSCOPIC EXAMINATION

EPITHELIAL CELLS

8-10

0-5

/HPF

METHOD : MICROSCOPIC EXAMINATION

CASTS

NOT DETECTED

METHOD : MICROSCOPIC EXAMINATION

CRYSTALS

NOT DETECTED

METHOD : MICROSCOPIC EXAMINATION

BACTERIA

NOT DETECTED

NOT DETECTED

METHOD : MICROSCOPIC EXAMINATION

YEAST

NOT DETECTED

NOT DETECTED

METHOD : MICROSCOPIC EXAMINATION

Interpretation(s)

The following table describes the probable conditions, in which the analytes are present in urine

Presence of	Conditions
Proteins	Inflammation or immune illnesses
Pus (White Blood Cells)	Urinary tract infection, urinary tract or kidney stone, tumors or any kind of kidney impairment
Glucose	Diabetes or kidney disease
Ketones	Diabetic ketoacidosis (DKA), starvation or thirst
Urobilinogen	Liver disease such as hepatitis or cirrhosis
Blood	Renal or genital disorders/trauma
Bilirubin	Liver disease
Erythrocytes	Urological diseases (e.g. kidney and bladder cancer, urolithiasis), urinary tract infection and glomerular diseases
Leukocytes	Urinary tract infection, glomerulonephritis, interstitial nephritis either acute or chronic, polycystic kidney disease, urolithiasis, contamination by genital secretions
Epithelial cells	Urolithiasis, bladder carcinoma or hydronephrosis, ureteric stents or bladder catheters for prolonged periods of time
Granular Casts	Low intratubular pH, high urine osmolality and sodium concentration, interaction with Bence-Jones protein
Hyaline casts	Physical stress, fever, dehydration, acute congestive heart failure, renal diseases

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Calcium oxalate	Metabolic stone disease, primary or secondary hyperoxaluria, intravenous infusion of large doses of vitamin C, the use of vasodilator naftidrofuryl oxalate or the gastrointestinal lipase inhibitor orlistat, ingestion of ethylene glycol or of star fruit (Averrhoa carambola) or its juice
Uric acid	arthritis
Bacteria	Urinary infection when present in significant numbers & with pus cells.
Trichomonas vaginalis	Vaginitis, cervicitis or salpingitis

Reference:

Henry's Clinical Diagnostics and Management by Laboratory Methods 21st edition (pg 410).

25 - HYDROXYVITAMIN D(VITAMIN D TOTAL),SERUM

25 - HYDROXYVITAMIN D

57.5

Assay specific category :- ng/mL

Deficiency : <10

Insufficiency : 10-30

Sufficiency : 30-100

Toxicity : >100

METHOD : CLIA

****End Of Report****Please visit www.agilusdiagnostics.com for related Test Information for this accession

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CONDITIONS OF LABORATORY TESTING & REPORTING

1. The result relate only to the items tested in our laboratory.
2. For tests performed on specimens received from other locations, it is presumed that the specimen belongs to the patient named or identified, such verification having been carried out at the point of generation of the said specimen.
3. The test results reported are for the information of the referring doctor only.
4. The technical note given below a test heading is a general comment on the TEST.NO
5. A requested test might not be performed for the following reasons:
 - a. Specimen quantity is insufficient (in-adequate collection/spillage in transit)
 - b. Specimen quality unacceptable (hemolyzed/clotted/lipemic/etc).
 - c. Incorrect specimen type.
 - d. Test may be cancelled either on request of patient or doctor, it is expected that a fresh specimen will be sent for the purpose of reporting on the same parameter.

6. A test requested might yield "INVALID RESULT" for various technical reasons. It is expected that a fresh specimen will be sent for the purpose of reporting on the same parameter..
7. In the event of unforeseen circumstances, unavailability of kits, failure of test runs, instrument breakdown, DDRC AGILUS PATHLABS LIMITED will make all efforts to minimize the delay in reporting..
8. Any query from referring doctor pertaining to this report should be directed to concerned HOD at DDRC AGILUS PATHLABS LIMITED.

DDRC AGILUS PATHLABS LIMITED

DDRC agilus Tower, G-131, Panampilly
 Nagar, Ernakulam 682036

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